

Safety screening completion in patients initiating immune response mediator therapy for rheumatology, dermatology, and gastroenterology conditions: A multisite prospective analysis

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PURPOSE

Despite the recommendation that patients initiating immune response mediator therapies for chronic autoimmune conditions be screened for tuberculosis (TB) and hepatitis B virus (HBV) prior to initiation, **previous studies have found low screening rates.**

This analysis **assessed TB and HBV screening completion rates at the time of immune response mediator initiation and reasons for incomplete screening** in health-system specialty pharmacies.

METHODS AND POPULATION

Design	Multisite (12 sites), prospective, observational cohort
Sample	- Inclusion: Adult patients initiating a medication with risk for TB or HBV reactivation indicated for a rheumatology, dermatology, or gastroenterology condition between March 1 and May 31, 2023 - Exclusion: Biologic medication use within 1 year prior to index referral date, referral from a non-health system provider, lost to follow-up before screening or treatment initiation
Data sources	- Electronic health record was reviewed for screening documentation - Fill data from health system specialty pharmacy claims or electronic health record data - Data independently collected from each site and imported into shared data warehouse hosted at Vanderbilt University Medical Center

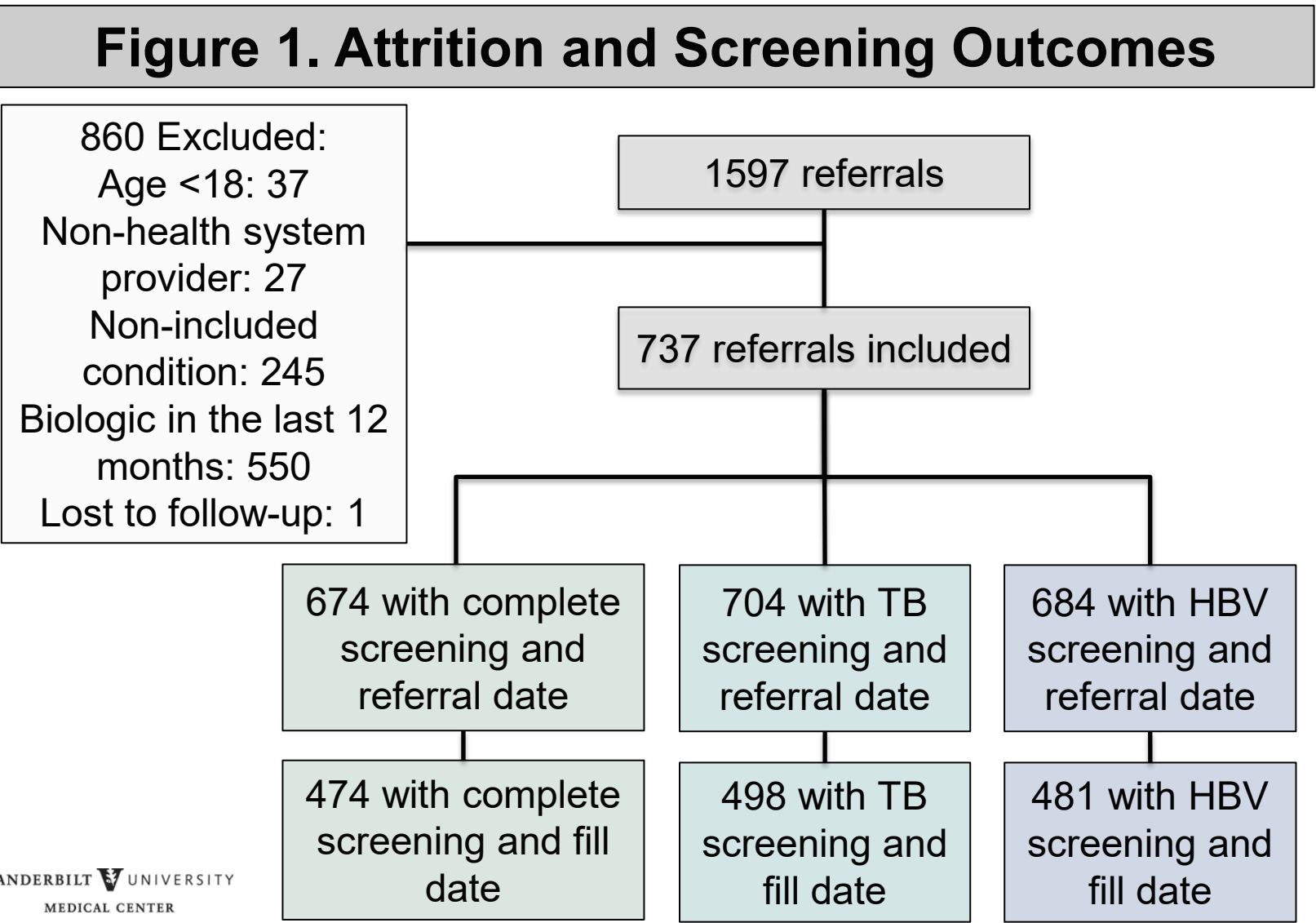


Table 1. Patient Demographics	
Demographic	N=737 n (%)
Age- mean (SD)	48 (16)
Gender- Female	469 (64)
Race	
White	492 (67)
Black/African American	95 (13)
Clinical Area	
Dermatology	214 (29)
Gastroenterology	114 (16)
Rheumatology	409 (56)

CONCLUSIONS

Most patients with a health-system specialty pharmacy referral are screened for TB and HBV prior to treatment.

Omitted safety screening is commonly driven by the prescribing provider.

RESULTS

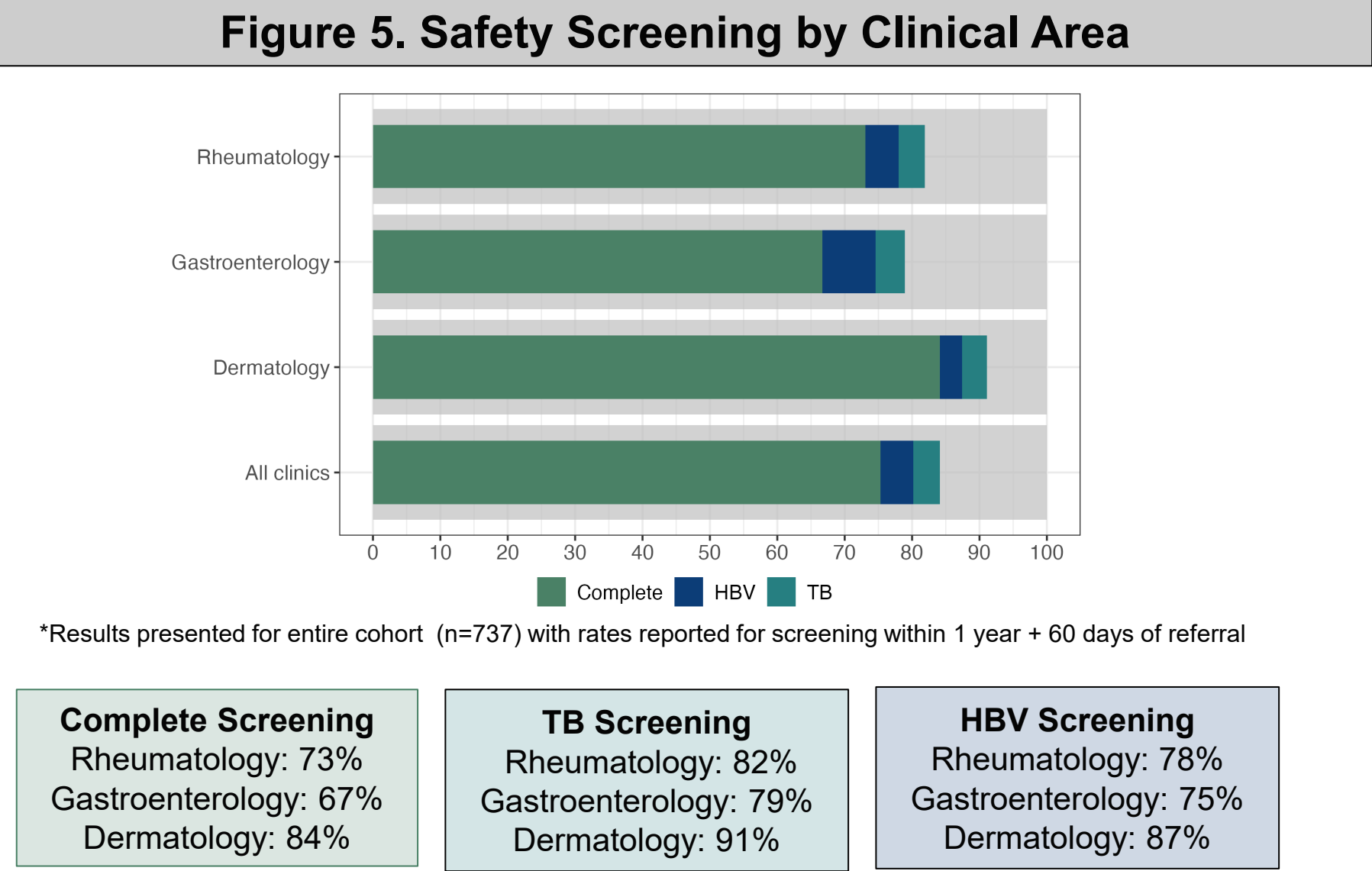
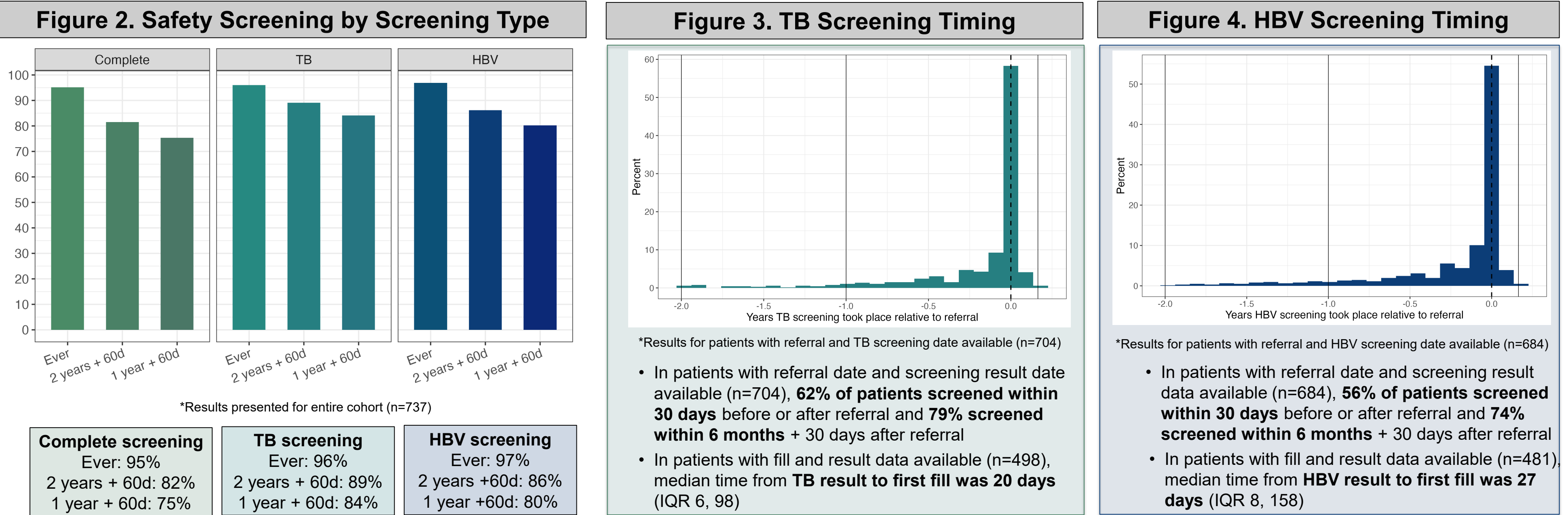


Table 2. Reasons not Screened by Clinical Area				
Reason not screened	Dermatology n (%)	Gastroenterology n (%)	Rheumatology n (%)	Overall n (%)
TB screening				
Provider driven	1 (11)	1 (33)	9 (53)	11 (38)
Patient driven	1 (11)	0 (0)	3 (18)	4 (14)
Lab error	2 (22)	0 (0)	0 (0)	2 (7)
Noninitiation	2 (22)	0 (0)	0 (0)	2 (7)
Unknown	3 (33)	2 (67)	5 (29)	10 (34)
HBV screening				
Provider driven	4 (33)	7 (70)	13 (65)	24 (57)
Patient driven	3 (25)	0 (0)	2 (10)	5 (12)
Noninitiation	2 (17)	0 (0)	0 (0)	2 (5)
Unknown	3 (25)	3 (30)	5 (25)	11 (26)

Reasons for lack of screening are presented for patients who never had any screening in the entire cohort (n=737)

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